



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : صباح سلومه عجاب لطفي
المحافظة : القاهرة الحي / المدينة : حلوان
رقم البطاقة : 28306062300407 رقم التليفون : 01126015120
العمر (تاريخ الميلاد) : 06-06-1983 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي حوض [11107] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: صباح سلومة عجاب لظف
National ID: 28306062300407



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي MRI of the pelvis.

الوقت :

---:--

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاثنين



02/28/2026

MDT FORM

Name:	Sabah Sioma Egab Lotfe	MRN:	4083
Age:	43 Y	ID:	28306062300407
Menopausal Sta:	Premenopausi	Phone:	01091512476
NRS SCORE:		Marital Status:	
BMI:		Family History:	No
CO Morbidity:		Reason Of MDT:	Systemic treatment

Clinical Data:

BX: IDC
ER: 7/8 PR: 2/8 HER2: 1+ KI67:

R

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Sonomammography:

Mets Workup:

History:

A 44-year-old female diagnosed in 2019 with right breast cancer. She received NACT in the form of AC followed by Taxol, then underwent right BCS with SLNB followed by PORT. She was subsequently maintained on adjuvant endocrine therapy with aromatase inhibitor (AI) plus Zoladex for four years. In 2024, she developed metastatic disease involving the bone and lungs. She was treated with palbociclib in combination with Fulvestrant for six months; however, bone scan showed disease progression. Accordingly, treatment was shifted to (Aromasin) plus (Afinitor) with (Zoladex), Received for 7 months

Bone Scan 29/12/2025 : SD of multiple bony lesions

CT Brain: Rt frontal and Rt temporal SOL with extracranial and intraorbital extension, Did Brain MRI pending results.

O/E: Multiple new cervical LN's. For MDT to shift ttt.

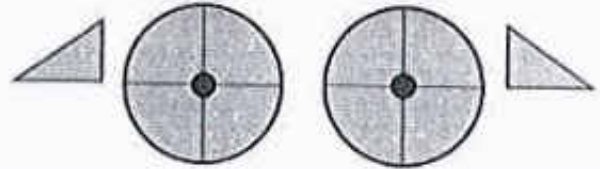
Recommendations:

Oral navelbine + Zometa

MDT Outcome & Plan

FOR REBIOPSY

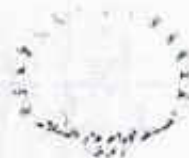
MEANWHILE TO START TAXOTER SINGLE AGENT



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاني حريت					



Patient Name: Sabah Sallouma Agab Lotfy	Date : 22/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 3668/2024	Pat. ID: 97876

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

****known case of right conservative breast surgery on hormonal treatment:**

1. Chest:

- Right breast LIQ surgical clip with no sizable masses.
- Few subcentemetric right axillary level I lymph nodes.
- Few prominent bilateral internal mammary lymph nodes, largest on the right side measures about 8mm.
- Enlarged left axillary lymph nodes largest 2.9x1.5 cm.
- Prominent right cardiophrenic lymph node, measures about 9 x6 mm.
- Multiple bilateral scattered pulmonary nodules, the largest pleural based lingular inferior segment measuring 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Side finding: Enlarged left thyroid lobe showing partially cystic nodule, for ultrasonographic assessment.

2. Abdomen and pelvis:

- The liver is average in size showing few bilobar hepatic focal lesions largest is noted at segment V/VI measuring about 1.3 x 1.3 cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- Diffuse sclerotic osseous patchy areas and foci involving sternum, multilevel dorso-lumbar vertebrae, left sacrum, right acetabulum anterior column as well as right greater trochanter.
- Right internal mammary retrosternal extra-osseous soft tissue mass lesion, measuring about 4.5 x 3 cm seen entangling the right internal mammary vessels, associated with significant erosive changes of the related sternal border.



Patient Name: Sabah Sallouma Agab Lotfy	Date : 22/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 3668/2024	Pat. ID: 97876

- L1 schmorl's node /central depression.

OPINION:

****known case of right conservative breast surgery on hormonal treatment.**

- ***Multiple bilateral scattered pulmonary nodules, ?? metastatic nature.***
- ***Few bilobar hepatic focal lesions, Likely metastatic, for triphasic CT assessment.***
- ***Diffuse sclerotic osseous patchy areas involving sternum, multilevel dorso-lumbar vertebral and pelvic bones with right retrosternal extra-osseous soft tissue mass lesion, mostly of metastatic nature.***

Dr.Menna Salem, MSc.

Dr. Walid Mohamed, M.D.



Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: صباح سلومة عجاب لطفي
Patient's I.D.: 28306062300407
Age: 41 Y.

Pathology Number: 252 /2024
Specimen Received Date: 30 / 07 /2024
Date Report Issued: 12/ 08 /2024

Clinical data: H/O right cancer breast with CBS.
Nature of the specimen: Left axillary lymph node, U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of nodal tissue showed effacement of architecture and infiltration by malignant tumor formed of trabeculae and groups as well as single and signet ring forms of markedly anaplastic ductal epithelial cells.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain. Tissue sections from paraffin block were used for quantitative detection of HER2 amplification using Ventana Benchmark Ultra System autostainer & VENTANA INFORM HER2 Dual DNA Probe Cocktail Assay.

IMMUNOHISTOCHEMISTRY RESULTS:

Estrogen receptors: moderate positive nuclear staining in most of tumor cells.
Progesterone receptors: weak positive nuclear staining in <1% of most of tumor cells.
Her-2/neu: Weak to moderate incomplete membrane staining in >10% of tumor cells (score 1+).
Ki67: Positive nuclear stain in average of ±80% of tumor cells.

DIAGNOSIS: Left Axillary Lymph Node, U.S. Guided Tru Cut Biopsy;
Metastatic Carcinoma, Compatible With Mammary Duct Origin.

- **Positive** For Estrogen Receptors, Score 7/8.
- **Negative** For Progesterone Receptors, Score 2/8.
- **Low Positive** For Her-2/neu over-expression (Score 1+).
- **80%** Ki67 Proliferative Index.

PROF. DR. AHMED ABDEL AZIZ
DR. PASSANT ESSAM SHIBEL

اتفاقية
1.9.10.12.27
ن.ال.ر.ص

Name : Sabah Salouma Ajeeb Lotfy
Branch : Dokki

Date : 05-09-2025 10:53
Medical No. : 195034

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 44-year-old lady, with history of metastatic breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Considering the previous bone scan dated 14/10/2024, the current study revealed:
 - Newly developed active lesions at the skull, proximal shaft of right humerus, multiple dorso-lumbar vertebrae (with progressed old ones), pelvic bones and trochanteric region of right femur.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Metastatic osseous lesions with progressive course evidenced by multiple newly-developed deposits.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



9/8/26, 4:50 PM

code:02 - Name_A MDT - Name_E MDT

د. سناء
عبدالله
عبدالله

9-1



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
سجل
الطب

09/08/2026

MDT FORM

Name:	Sahar Siamra Egab Lode	MRN:	4083
Age:	43 Y	ID:	26306062300407
Menopausal Sta		Phone:	01126025820
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve
CO Morbidity:	free	Reason OF MDT:	dp

Clinical Data:

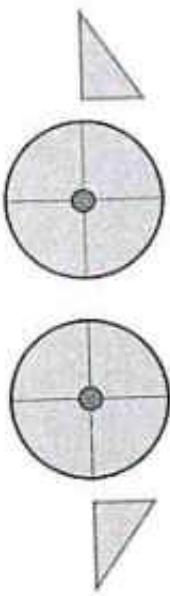
BX:

ER: 7/8 PR: 2/8 HER2: +1 KI67:

R

Sonomammography:

Mets Workup:



T N M

History:

A 44-year-old female diagnosed in 2019 with right breast cancer. She received NACT in the form of AC followed by Taxol, then underwent right BCS with SLNB followed by PORT. She was subsequently maintained on adjuvant endocrine therapy with aromatase inhibitor (AI) plus Zoladex for four years. In 2024, she developed metastatic disease involving the bone and lungs. She was treated with palbocicli in combination with fulvestrant for six months, however, bone scan showed disease progression. Accordingly, treatment was shifted to afnitor/aromasin till 3/2025. complained of headache and blurring of vision >> mri brain skull bony mets extraxial sol nec wcl 3/2025
new ct >> ad with HFLs
new biopsy from HFL 7/2026 >> met carcinoma of breast origin grade 3. er 6/8 pr0/8 her2-ve
then shifted to taxotere received bc with bone scan showed PD
ECOG >> 3
PT complain of severe pain at RT hemipelvis region + lumbar region
Recommendations:

MDT Outcome & Plan

For palliative Radiotherapy
If PS improves --> For oral Navelbine + Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Signature of Sahar Siamra Egab Lode

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Patient's Name: Sabah Saloma Ajeeb Lotfi
Date: 2026-07-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- History of breast cancer
- compared to last study dated 26/4/2026 current study revealed:

CHEST

- Clear both breasts and right axilla.
- Still seen left axillary enlarged suspicious LNs for US correlation
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules of average size 2-3mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

ABDOMEN AND PELVIS: -

- The liver is of average size with homogenous CT texture with decrease in size of the previously mentioned bilobar multiple hepatic focal lesions with peripheral enhancement largest is seen at segment III measuring 20x19mm compared to 2.9x3cm at previous study
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

THE EXAMINED SKELETON

- Still seen multiple scattered lytic and sclerotic mainly sclerotic bony lesions are seen along the axial and appendicular skeleton denoting metastasis.

Opinion:

- Regressive course of hepatic focal lesions
- Stationary rest of study.

DR/NERMEEN RAMADAN.MD

Name : Sabah Saloma Ajeeb Lotfi

Branch : Dokki

Ref.Physician Prof: Goustav Roussey Egypt

Date : 23-07-2026 04:12

Medical No. : 423082

WHOLE-BODY BONE SCINTIGRAPHY REPORT

Indication: Follow-up of metastatic breast carcinoma.

Comparison: Prior bone scan dated September 5, 2025.

PROCEDURE

Planar anterior and posterior whole-body images following IV 99mTc-MDP.

FINDINGS

Skull: Interval regression of calvarial lesions compared to prior study.

Spine: Significant progression with multiple new intensely active osteoblastic lesions throughout cervical, thoracic, and lumbar spine.

Pelvis: Progressive disease with new foci involving both iliac bones, acetabula, pubic rami, and sacrum.

Upper Extremities: New prominent lesions in proximal humeri bilaterally (right > left).

Lower Extremities: New focal uptake at right femoral trochanter and proximal shafts.

Kidneys & Bladder: Physiological excretion.

Patient Name	صباح سلوة عبد	Case No.	276242
Sex/Age	Female 43 Y	Patient No.	20691
Registered Date	13/6/2026 09:48:33 AM	Referred By Dr.	Interventional Radiology
Validation Date	05/7/2026 01:33:58 PM	Branch	Gustave Roussy
Reporting Date	13/7/2026 10:59:27 AM	Pathology No.	1859/26

Clinical Data: Hepatic focal lesion in a patient with previous history of breast cancer

Nature of Specimen: Tru-cut biopsy, collected on 13/06/2026, at 10:20 am. Cold ischemia time is 20 seconds.

Technique:

- Technique:**
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- FR clone (1E2), PR clone (SP1), Her2 clone (4B5) & GATA3 clone (L50-823).

Microscopic examination:

Microscopic examination:
-Histopathological examination reveals cores of hepatic tissue showing metastatic breast carcinoma, grade 3.

Results:

- * IPT (All controls showed appropriate reactivity):
- GATA3: Strong diffuse positive staining in tumor cells.
- ER: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).

Conclusion:

Hepatic focal lesion, Tru-cut biopsy:

- Consistent with metastatic deposits from breast carcinoma, grade 3.

IPT:

- GATA3: Positive.
- ER: Positive (score 6/8).
- PR: Negative (score 0/8).
- Her2: Ultra-Low (score 0+).

^aScoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2,



intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+, based on circumferential membrane staining that is incomplete and/or weak/moderate and within $\geq 10\%$ of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within $<10\%$ of the invasive tumor cells. IHC 3+, based on circumferential membrane staining that is complete, intense $>10\%$ of tumor cells.

Signature _____

Dr. Walaa Ibrahim Ghoneim, MD.



جوستاف
رؤسسي
رؤسسي
رؤسسي

Patient's Name:

Sabah Saloma Ajeeb Lotfi

Date:

2026-07-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Clear operative bed in the breast with no evidences of either residual and /or recurrent neoplastic lesions seen.
- Normal Both axillae.
- Hyperinflated both lungs.
- Few enhancing tiny subpleural pulmonary nodules are seen in the medial and in the lateral segment of left lower pulmonary lobes with the largest one measures about 5.4mm in diameter.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Foci of osseous sclerotic lesions and to lesser extent lytic lesions are seen scattering in the skeleton.
- Order line size of the liver,measuring about 15.8 cm, showing a homogenous parenchymal attenuation and enhancement pattern. A tiny hypovascular subcapsular focal lesion is seen in segment VIII of the right lobe and measures about 16x16mm in dimensions.
- No other sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Border line sized spleen,measuring about 13 cm. No focal lesions seen inside.
- Relatively thickened left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys(apart from a tiny cortical cyst in the lower pole of left kidney and measures about 16x13mm in dimensions), right adrenal gland,aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.



Patient's Name:

Sabah Saloma Ajeeb Lotfi

Date:

2026-07-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- Clear operative bed in the breast with no evidences of either residual and /or recurrent neoplastic lesions seen and normal Both axillae.
- Hyperinflated both lungs with few tiny enhancing metastatic subpleural pulmonary nodules bilaterally, foci of osseous sclerotic lesions and to lesser extent lytic lesions scattering in the skeleton, denoting osseous deposits, a tiny hypovascular subcapsular focal lesion in segment VIII of the right hepatic lobe, likely to be a metastasis and relatively thickened left adrenal gland, denoting a metastasis or a hyperplasia.

Prof/Hanan El-Ahwal.

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مركز
التصوير
الاشعاعي
بمصر

Patient's Name:

SABAH SALOMA AGAB LOTFY

Date:

2026-04-26

Referred by:

Prof.Dr. CT CAP IV

Hospital:

Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-

Clinical History:

- **History of breast cancer underwent lumpectomy, compared to last study dated 9/7/2025 current study revealed:**

Findings:

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs are seen largest is measuring 2.2x2.5cm.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules of average size 2-3mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-

Findings:

Patient's Name: SABAH SALOMA AGAB LOTFY
Date: 2026-04-26
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

- The liver is of average size with homogenous CT texture with bilobar multiple hepatic focal lesions with peripheral enhancement largest is seen at segment III measuring 2.9x3cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Multiple scattered lytic and sclerotic mainly sclerotic bony lesions are seen along the axial and appendicular skeleton denoting metastasis.

Opinion:

- **Newly developed hepatic and bony metastasis as detailed.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban



Patient's Name:

SABAH SALOMA AGAB LOTFY

Date:

2026-04-26

Referred by:

Prof.Dr. CT CAP IV
Gonstave Roussey International Egypt

Hospital:

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-

Clinical History:

- **History of breast cancer underwent lumpectomy, compared to last study dated 9/7/2025 current study revealed:**

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OPINION:

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- Left axillary enlarged suspicious LNs.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-

Findings:

Patient's Name: SABAH SALOMA AGAB LOTFY
Date: 2026-04-26
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

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Opinion:

- **Newly developed hepatic and bony metastasis as detailed.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

2. 12

**GUSTAVE /
ROUSSY**
INTERNATIONAL
ALGERIE

Name : SABAH SALOMA OGAAB	Date : 14-02-2026 04:54
Branch : Portable	Medical No. : GRI-28306062300407
Ref.Physician Prof : MRI BRAIN +C	Ins.Company :

FOLLOW UP MRI OF THE BRAIN

TECHNIQUE:

Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast T1 WIs

FINDINGS:

- Few right and left dural based extra-axial space occupying lesions are seen in the right fronto-temporal, right high parietal and left fronto-parietal region with all of them invading the overlying skull bone and scalp with contour bulge impressive of metastatic brain and skull deposits. They are seen eliciting isointense signal in T1, mixed iso and high signal in T2 and FLAIR WIs with post contrast enhancement.



- The largest lesion is seen dural based at the right fronto-temporal region measuring 6.3 x 3 x 4.6 cm at its maximal CC, TV and AP diameters.
- Symmetrical appearance of the ventricular system.
- No intra or extra axial areas of fresh blood signal intensities or its degradation products.
- The brain stem and cerebellum are normal.

OPINION:

- No available previous studies showing few bilateral metastatic deposits in skull bone and extra-axial brain deposits.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

MDT FORM

MDT FORM

MDT FORM

MDT FORM

<u>Name</u>	Dr. [Signature]	<u>MRN</u>	283060623004
<u>Age</u>	43 yrs	<u>ID</u>	277N/C.C.C
<u>Menopausal status</u>	perimenopausal	<u>Phone</u>	01091512476
<u>NRS SCORE</u>		<u>Marital status</u>	
<u>BMI</u>		<u>Family history</u>	-ve
<u>CO morbidity</u>	No	<u>REASON OF MDT</u>	

Clinical Data

<u>BX:</u> <u>IOC</u>			
<u>ER:</u> <u>7/8</u>	<u>PR:</u> <u>2/8</u>	<u>HER 2:</u> <u>1+</u>	<u>Ki67:</u>
<u>Sonomammography:</u>			
<u>Mets Workup:</u> (See scan 12/9/2023) → progressive Cans with newly developed deposits.			

Recommendations:

2019 11 Break IDC (EE 710 SR 248 42 1- K67857.)

reduced water $\rightarrow$ RF BLS $\rightarrow$ PPS $\rightarrow$ TBSA / XI

2021 while on 2010s/11E → base of W mth → AX W br
IX (see 7/8 PR 2/8 H-2 1+) → 12 line Foshan + JTBanc

MDT OUTCOME & PLAN

PLAN (read 6 only but only one base sea in 10/2/24) → 179/2/25 Base sea

Consider Shift to ZLack / Aromatic / Alkyl.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
5 Dec 1968	10/11/68	10/11/68	10/11/68	10/11/68	10/11/68

2012/10/16

MDT FORM 22 / 15 / 2024

Name	صباح سحر	MRN	61726
Age	41	National ID	
Menopausal status	premenopausal	Phone	01031512476
Marital status	married	Wt., Ht.	
Co morbidities	nil	Family history	-ve

Clinical Data

Metastatic CA

Bx: mets CA compatible e'mammary duct

ER: +ve PR: -ve HER2: t+

7/8 2/8 80%

Sonomammography: Left axillary level I & II LNs

4x1.6cm e' cortical thickness 1.2

LT Breast: free

RT Breast: clear axillary

Mets Worked: Bone scan -> osseous lesions in

sternum, L V1, L V4

CT CAP: -> soft tissue lesion at 3rd L thoracic chondral

5.4x3.3 cm, & axillary LNs, few bilateral small pleural nodules

few pleural thickening

Recommendations and further workup:

Follow up in 2019. RT IDC G2 ER 8/8 PR 8/8 Her 2-ve Ki67 20%

NACT ACx4, total 12 wks underwent at BCS & SLNB by PTIN1b (2/14)

received PORTH (breast DC LNs) on AI zoladex for 4 years.

then C/O of axillary recurrence.

Bx: IDC, ER 9/8 PR 2/8 HER2 -1

In view of relapse of on city Zoladex-AI

MDT OUTCOME & PLAN

for - 2nd line Fasldeg - Zoladex - Ibrance

+ Zomita 13m

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

psychiatry

MEDICAL ONCOLOGY

SURGERY

Name : Sabah Salouma Ajeeb Lotfy

Date : 05-09-2025 10:53

Branch : Dokki

Medical No. : 195034

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 44-year-old lady, with history of metastatic breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Considering the previous bone scan dated 14/10/2024, the current study revealed:
 - Newly developed active lesions at the skull, proximal shaft of right humerus, multiple dorso-lumbar vertebrae (with progressed old ones), pelvic bones and trochanteric region of right femur.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Metastatic osseous lesions with progressive course evidenced by multiple newly-developed deposits.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



PATHOLOGY REPORT

Name: صباح سلومه عجاب لطفي
Patient's I.D.: 28306062300407
Age, 41 Y.

Pathology Number: 252 /2024
Specimen Received Data: 30 / 07 /2024
Date Report Issued: 13/ 08 /2024

Clinical data: H/O right cancer breast with CBS.

Nature of the specimen: Left axillary lymph node, U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of nodal tissue showed effacement of architecture and infiltration by malignant tumor formed of trabeculae and groups as well as single and signet ring forms of markedly anaplastic ductal epithelial cells.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain. Tissue sections from paraffin block were used for quantitative detection of HER2 amplification using Ventana Benchmark Ultra System autostainer & VENTANA INFORM HER2 Dual DNA Probe Cocktail Assay.

IMMUNOHISTOCHEMISTRY RESULTS:

Estrogen receptors: moderate positive nuclear staining in most of tumor cells.
Progesterone receptors: weak positive nuclear staining in <1% of most of tumor cells.
Her-2/neu: Weak to moderate incomplete membrane staining in >10% of tumor cells (score 1+).

Ki67: Positive nuclear stain in average of $\pm 80\%$ of tumor cells.

DIAGNOSIS: Left Axillary Lymph Node, U.S. Guided Tru Cut Biopsy:

Metastatic Carcinoma, Compatible With Mammary Duct Origin.

- **Positive** For Estrogen Receptors, Score 7/8.
- **Negative** For Progesterone Receptors, Score 2/8.
- **Low Positive** For Her-2/neu over-expression (Score 1+).
- **80% Ki67** Proliferative index.

PROF. DR. AHMED ABDEL AZIZ
DR. PASSANT ESSAM SHIBEL

أحمد عبد العزيز
علاء شيبيل
13/08/2024
(مض)

جمهورية مصر العربية
بطاقة تعريف الشخصية



صباح
سلومة عجاب لطفى
٣٤ ش حسين زكى جزيرة الذهب
الجيزة - الجيزة

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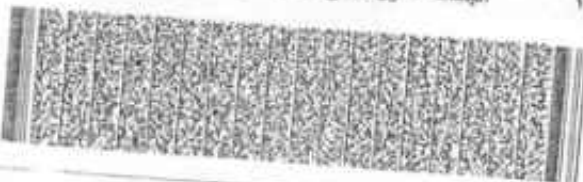
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بدون عمل

النسب مسلمة
السيد محمد عبدالله عطيه
الهيئته سارية حتى ٢٠٢٥/١٢/١٦



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مفت ملين ش



Name: Sabah Salouma Ogab

Date: 28/8/2024

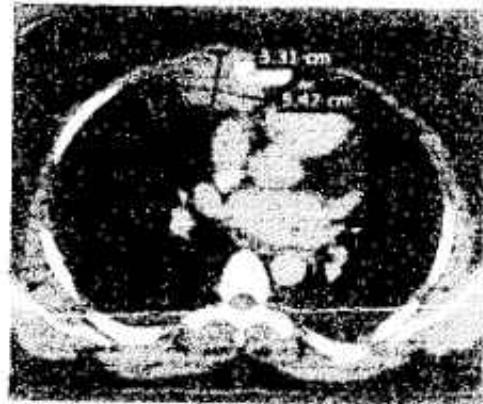
POST-CONTRAST CT SCAN OF THE CHEST:-

**** Reported case of breast cancer underwent neoadjuvant chemotherapy with CBS:**

**** Examination is degraded by the limited number of chest cuts hindering proper assessment:**

Findings:

- Lobulated soft tissue lesion seen implicating the right parasternal region medial to the 3rd and 4th costo-chondral junctions, measuring about 5.4x3.3 cm in axial dimensions, seen abutting the sternal body and the right internal mammary vessels.
- Multiple enlarged left axillary lymph nodes, some of them showing globular outline, the largest measuring about 4x1.6 cm in axial dimensions.
- Few bilateral small predominantly subpleural and fissural nodules, the largest measuring about 4 mm in diameter.
- Lingular parenchymal band.
- No consolidation, cavitation or bronchiectatic changes.
- Few prominent prevascular and paratracheal lymph nodes, with subcentimetric right cardiophrenic lymph node.
- No pleural or pericardial collections.
- No gross cardiac abnormality.
- Right breast dense metallic structure, likely small surgical clip.



Dr/ Aya Hussein, MD

Name: Sabah Salouma Ogab

Date: 28/8/2024

CECT SCAN OF THE ABDOMEN AND PELVIS: -

**** Known case of cancer breast underwent neoadjuvant chemotherapy with CBS:**

**** Examination is degraded by limited number of abdominal cuts hindering proper parenchymal assessment:**

Findings:

- Mildly enlarged liver showing smooth outline with homogenous parenchymal texture. No sizable focal lesions. No biliary radicles dilatation.
- Normal CT appearance of the spleen, kidneys, adrenal glands and pancreas.
- Average filled urinary bladder with no sizable mass lesions or diverticular outpouchings.
- Few prominent bilateral iliac lymph nodes. No sizable pathologically enlarged abdominal lymph nodes.
- No ascites.

**** For correlation with clinical data and previous unavailable films.**

Dr/ Aya Hussein, MD